

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

February 21, 2023

MEMORANDUM FOR: (b)(6),(b)(7)(C),(b)(7)(F) DHSc, FACHE
Assistant Director (b)(6),(b)(7)(C),(b)(7)(F)
ICE Health Service Corps

THROUGH: (b)(6),(b)(7)(C),(b)(7)(F) MD (b)(6),(b)(7)(C),(b)(7)(F)
Deputy Assistant Director of Clinical Services/Medical Director
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F) MS
Chief of Staff (b)(6),(b)(7)(C),(b)(7)(F)
ICE Health Service Corps

CDR (b)(6),(b)(7)(C),(b)(7)(F) PA-C (b)(6),(b)(7)(C),(b)(7)(F)
Chief, Investigations Unit
ICE Health Service Corps

FROM: CDR (b)(6),(b)(7)(C),(b)(7)(F) RDH (b)(6),(b)(7)(C),(b)(7)(F)
Program Manager/Investigator
ICE Health Service Corps

SUBJECT: Mortality Review – Report of Findings
Sanchez PEREZ, Luis (aka HERNANDEZ-Cabrera, Mauricio),
(b)(6),(b)(7)(C)

Executive Summary:

On July 15, 2020, U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of ICE detained noncitizen Sanchez PEREZ, Luis, aka HERNANDEZ-Cabrera, Mauricio, (b)(6),(b)(7)(C), hereinafter referred to as Mr. PEREZ. Mr. PEREZ, a 46-year-old Guatemalan male, was in ICE custody from January 29 to July 15, 2020, and assigned to Catahoula Correctional Center (CCC) in Harrisburg, Louisiana (LA), on the date of his death. An autopsy ruled Mr. PEREZ's cause of death resulted from complications of diabetes mellitus (DM), requiring extensive hospitalization for recurrent infections, renal

Mortality Review –

Sanchez PEREZ, Luis (aka HERNANDEZ-Cabrera, Mauricio), (b)(6),(b)(7)(C)

Page 2 of 9

(kidney) and liver failure, with contributions of hypertensive (elevated blood pressure) atherosclerotic cardiovascular disease.

On January 25, 2020, Enforcement and Removal Operations (ERO) Criminal Alien Program (CAP) encountered Mr. PEREZ at Hamilton County Jail in Chattanooga, Tennessee, after his arrest for driving under the influence and without a driver's license. ERO CAP determined Mr. PEREZ illegally entered the U.S. and issued him an ICE detainer.

On January 29, 2020, ERO Chattanooga served Mr. PEREZ a Notice to Reinstate Prior Order, form I-871, booked him into ICE custody, and transferred him to Etowah County Jail (ECJ) in Gadsden, Alabama.

Between January 30 and February 26, 2020, ICE ERO detained Mr. PEREZ at the following facilities: ECJ (six days), LaSalle ICE Processing Center (LIPC) in Jena, LA (less than 12 hours); Winn Correctional Center in Winnfield, LA (19 days); and CCC (less than four hours).

On February 26, 2020, a CCC advanced practice provider (APP) evaluated Mr. PEREZ, noted his clinical presentation warranted a higher-level of care, and ordered his transfer to Oschner Louisiana State University Health Monroe Medical Center (OLSUH-Monroe) in Monroe, LA, via emergency medical services (EMS).

Between February 26 and July 13, 2020, OLSUH-Monroe admitted Mr. PEREZ and treated him for numerous conditions, including congestive heart failure, DM, recurrent infections, and bilateral pneumothoraces (the presence of air or gas in the cavity between the lungs and chest wall, causing a collapsed lung); however, his condition declined throughout his hospitalization.

On July 14, 2020, an OLSUH-Monroe medical doctor (MD) evaluated Mr. PEREZ, noted the absence of his gag and corneal reflexes, and determined Mr. PEREZ suffered brain death.

On July 15, 2020, the MD informed Mr. PEREZ's family of his condition, and they decided to discontinue all medical care. At 11:45 a.m., the MD pronounced Mr. PEREZ deceased.

Mortality Finding:

As a result of Mr. PEREZ's death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures or practices are warranted; and identify issues requiring further study.

A mortality review committee (MRC) determined Mr. PEREZ's medical care at ECJ and WCC was not provided within the safe limits of practice. As collateral findings, the MRC determined

Mortality Review –

Sanchez PEREZ, Luis (aka HERNANDEZ-Cabrera, Mauricio), (b)(6),(b)(7)(C)

Page 3 of 9

Mr. PEREZ's January 30, 2020, chest x-ray results warranted additional imaging studies and evaluation, and ECJ and WCC did not appropriately address and treat Mr. PEREZ's elevated blood sugar (BS) and his self-reported history of DM.

The MRC determined the medical care provided at LIPC and CCC was within the safe limits of practice.

The MRC convened on January 20, 2023, and consisted of the following IHSC members:

(b)(6),(b)(7)(C),(b)(7)(F) MD, western regional clinical director, physician reviewer; Commander (CDR) (b)(6),(b)(7)(C),(b)(7)(F) physician assistant, eastern regional APP, APP reviewer; and CDR (b)(6),(b)(7)(C),(b)(7)(F) registered nurse (RN), central regional nurse manager, nurse reviewer.

The following summary of events was based on the following: 1) Mr. PEREZ's CCC, WCC, LIPC, and ECJ medical, emergency medical services (EMS) report, and hospital records; 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. PEREZ's CCC detention file; 5) Mr. PEREZ's autopsy report; 6) a remote-site review and staff interviews conducted by Fact Finder CDR (b)(6),(b)(7)(C),(b)(7)(F) at CCC and WCC in September 2020; and 7) applicable CCC and WCC policies and procedures, and ICE Detention Standards.

Summary of Events

On January 30, 2020, an ECJ emergency medical technician completed Mr. PEREZ's intake screening, documented he denied any medical conditions or complaints, and cleared Mr. PEREZ for general population (GP).

On February 2, 2020, an ECJ medical staff member (credential unknown) documented an order for Mr. PEREZ to receive twice daily BS checks for three days.

Between February 3 and February 5, 2020, ECJ medical staff documented Mr. PEREZ's BS results ranged from 159 to 247 milligrams per deciliter [(mg/dl); normal range is 80 mg/dl – 130 mg/dl, before a meal], and administered sliding scale insulin as indicated.

On February 6, 2020, ICE ERO transferred Mr. PEREZ to LIPC. An LIPC RN completed Mr. PEREZ's transient intake screening (an abbreviated intake screening for detainees who are in the facility less than twelve hours) and documented his denial of any medical conditions or taking medication. The RN reviewed and signed Mr. PEREZ's transfer summary.

On February 7, 2020, ICE ERO transferred Mr. PEREZ to WCC. A WCC RN completed Mr. PEREZ's intake screening and documented his self-reported history of DM, hypertension [(HTN); elevated blood pressure], and taking medications; although, he did not recall the names of his medications. In addition, the RN noted Mr. PEREZ's blood pressure (BP) as 173/94

Mortality Review –

Sanchez PEREZ, Luis (aka HERNANDEZ-Cabrera, Mauricio), (b)(6),(b)(7)(C)

Page 4 of 9

millimeters of mercury [(mmHg); normal less than 120/80 mmHg] and his BS as 145 mg/dl, ordered BP and BS checks for five days, and cleared Mr. PEREZ for GP.

On February 8, 2020, Mr. PEREZ presented to the medical clinic for “unclear complaints.” An RN evaluated Mr. PEREZ and documented his BP reading of 190/105 mmHg. The RN contacted an APP, who ordered STAT (immediate) doses of anti-hypertensive medications. The medications failed to reduce Mr. PEREZ’s BP, so the APP ordered Mr. PEREZ’s transfer to Winn Parish Medical Center (WPMC) in Winnfield, LA, for further evaluation.

At WPMC, an emergency department MD evaluated Mr. PEREZ for hypertensive crisis (severely elevated blood pressure) and admitted him for further evaluation and observation. Imaging and laboratory results showed findings consistent with nephrotic syndrome (kidney disorder that causes the excretion of too much protein in the urine), anasarca (a severe and generalized form of subcutaneous tissue swelling throughout the body), congestive heart failure [(CHF); failure of the heart muscle to pump blood through the body], severe anemia (deficiency of red blood cells), blood in his stool, and elevated BS levels. The MD ordered anti-hypertensive medications, to include a diuretic (medication used to promote increased urine production), two units of packed red blood cells via intravenous administration, and DM medication.

On February 9, 2020, a nephrologist evaluated Mr. PEREZ for proteinuria (excess protein in the urine), elevated BP, and Mr. PEREZ’s self-report of edema (swelling) to his upper and lower extremities for the past three to four months, and shortness of breath (SOB) with prolonged ambulation. The nephrologist ordered Mr. PEREZ a kidney ultrasound (results – the right kidney has mildly increased echogenicity and coarsening of its echotexture, characteristic of renal disease) and medication to treat his CHF.

On February 10, 2020, a gastroenterologist evaluated Mr. PEREZ for a positive fecal occult test (a test that shows blood in the stool) and a hemoglobin [(Hgb); iron containing oxygen transport in red blood cells] level of 10.2 grams per deciliter [(g/dl); normal range 13.7 g/dl – 17.5 g/dl]. Mr. PEREZ denied gastrointestinal (GI) symptoms. The gastroenterologist ordered an outpatient endoscopy (insertion of a tube into the body to examine the interior of a hollow organ or cavity) and colonoscopy (insertion of a tube into the rectum to examine the colon) after his follow-up and clearance by the nephrologist.

On February 11, 2020, WPMC discharged Mr. PEREZ from the hospital with a diagnosis of nephrotic syndrome, hyperglycemia (elevated blood sugar in the blood), anemia, and CHF. Upon his return to WCC, an RN assessed Mr. PEREZ, placed him in the medical housing unit (MHU) for close monitoring, and scheduled an appointment with an APP the next day.

On February 12, 2020, an APP evaluated Mr. PEREZ, ordered laboratory studies, specialty consultations, discharge medications to treat his medical conditions, and his continued

Mortality Review –

Sanchez PEREZ, Luis (aka HERNANDEZ-Cabrera, Mauricio), (b)(6),(b)(7)(C)

Page 5 of 9

monitoring in the MHU. In addition, the APP ordered a STAT dose of clonidine 0.1 mg by mouth for a BP reading of 163/96 mm/Hg.

February 13 to 24, 2020, WCC medical staff continued to monitor and treat Mr. PEREZ for his medical conditions, noting his condition as “stable” but high acuity.

On February 25, 2020, an APP evaluated Mr. PEREZ, and documented he walked with a slow gait and “appears sick.” During the exam, the APP noted bilateral diminished lung sounds, mild abdominal distention, 3 + pitting edema [(swelling when external pressure leaves a persistent depression in the tissue due to accumulation of excess fluid to the area); normal is less than 2+], fingernail clubbing (deformity of the fingernails associated mostly with heart and lung diseases), and jaundice (yellowish) skin. The APP adjusted Mr. PEREZ’s anti-hypertensive medication and “collaborated” with the MD and medical administration to transfer Mr. PEREZ to a facility that could provide him with a higher-level of care.

On February 26, 2020, ICE ERO transferred Mr. PEREZ to CCC. Upon arrival, a CCC APP evaluated Mr. PEREZ, noted severe swelling to his hands, jaundice skin, abnormal lung sounds, 3+ generalized pitting edema, and a BS of 577 mg/dl. Mr. PEREZ reported his swelling as normal, with symptoms starting two months prior, and associated with SOB. The APP ordered a STAT injection of ten units of regular insulin and transferred Mr. PEREZ to OLSUH-Monroe, via EMS. OLSUH-Monroe’s attending MD admitted Mr. PEREZ for exacerbation of CHF, SOB, and DM, with a history of other chronic care conditions (HTN and nephrotic syndrome).

Between February 27 and July 13, 2020, Mr. PEREZ remained hospitalized, and suffered multiple pulseless electrical activity[(PEA); condition characterized by unresponsiveness and impalpable pulse in the presence of sufficient electrical activity in the heart] events, bilateral pneumothoraces, and recurrent lung and blood infections. Despite some episodes of clinical improvement, Mr. PEREZ continued to decompensate

On July 14, 2020, an OLSUH-Monroe MD evaluated Mr. PEREZ and noted his long course of hospitalization events. During the exam, the MD found Mr. PEREZ with the absence of gag and corneal reflexes and determined Mr. PEREZ suffered brain death. The MD spoke with Mr. PEREZ’s son regarding the discontinuation of medical care, but no decisions were made.

On July 15, 2020, the MD, with the use of a Spanish interpreter, informed Mr. PEREZ’s family of his long battle with illness, PEA events, multiple infections, and organ failure. After the MD answered their questions, the family decided to discontinue Mr. PEREZ from all further medical care. At 11:09 a.m., the MD removed Mr. PEREZ from mechanical ventilation and administered medications to aid in his comfort. At 11:45 a.m., Mr. PEREZ’s heart monitor showed an absence of electrical activity. The MD confirmed he did not have audible heart or lung sounds and pronounced Mr. PEREZ deceased.

Mortality Review –

Sanchez PEREZ, Luis (aka HERNANDEZ-Cabrera, Mauricio), (b)(6),(b)(7)(C)

Page 6 of 9

The final autopsy report revealed Mr. PEREZ's cause of death was due to complications of DM requiring extensive hospitalization including recurrent infections, renal and liver failure, with contributions of hypertensive atherosclerotic cardiovascular disease. The coroner determined the manner of death as natural.

ICE detention standards used for this review: National Detention Standards (NDS) 2000 and Performance-Based National Detention Standards (PBNDS) 2011. The following are identified program strengths, weaknesses, and recommendations found during this review.

Strengths

Strengths identified during this investigation were: CCC medical staff immediately recognized Mr. PEREZ's poor clinical presentation, quickly activated EMS, and transferred him to the hospital on February 26, 2020.

Weaknesses

1. Clinical Care

- On January 30, 2020, ECJ completed Mr. PEREZ's chest x-ray for tuberculosis clearance. The APP reviewed his x-ray results and documented "essentially normal." However, the x-ray showed collateral findings, specifically scarring and linear atelectasis (could indicate collapse or closure of a lung resulting in reduced gas exchange), which warranted further imaging and evaluation.
- On February 7, 2020, a WCC RN completed Mr. PEREZ's health assessment. The clinical medical authority (CMA) did not review and sign the health assessment.
- On February 25 and 26, 2020, WCC nursing staff did not complete Mr. PEREZ's evening or early morning BS checks, respectively, and did not administer his morning dose of diabetes medication.
- Mr. PEREZ's medical conditions warranted oversight by the CMA or another MD; however, WCC APPs monitored and treated Mr. PEREZ without documented collaboration with an MD.
- WCC did not have written RN guidelines for addressing/referring abnormal findings.

Applicable standards of care for this finding:

- PBNDS 2011: Medical Care; section II. Expected Outcomes
- PBNDS 2011: Medical Care; section V.B. Designation of Authority
- PBNDS 2011: Medical Care; section V. M. Comprehensive Health Assessment
- PBNDS 2011: Medical Care; section V. U. Delivery of Medication

Mortality Review –

Sanchez PEREZ, Luis (aka HERNANDEZ-Cabrera, Mauricio), (b)(6),(b)(7)(C)

Page 7 of 9

2. Delay of Care

- On February 3 and 7, 2020, Mr. PEREZ self-reported a history of DM for which he took prescribed medications. However, ECJ and WCC medical staff did not immediately refer Mr. PEREZ to a higher-level provider for evaluation, delaying appropriate management of his elevated BS/DM.
- On February 5, 2020, an ECJ medical staff member (credential unknown) did not note Mr. PEREZ's order for BS checks or the results of those checks on his transfer summary, which caused a delay in Mr. PEREZ receiving his BS checks and sliding scale coverage with insulin at LIPC and WCC.
- On February 7, 2020, a WCC RN completed Mr. PEREZ's intake screening and health assessment. The RN did not refer Mr. PEREZ to a higher-level provider for his elevated BP and HR.
- On February 8, 2020, Mr. PEREZ presented to the medical clinic with an elevated BP reading indicating hypertensive urgency; however, the APP did not immediately order his transfer to the local hospital on the same date.
- On February 13, 2020, a WCC APP completed Mr. PEREZ's initial chronic care evaluation; six days after Mr. PEREZ reported a history of chronic care conditions.
- On February 24 and 25, 2020, an APP evaluated Mr. PEREZ and noted diminished lung sounds and worsening edema. Despite these findings, the APPs did not refer Mr. PEREZ to a higher-level provider, or the local hospital for further evaluation.

Applicable standards of care for this finding:

- NDS 2000: Medical Care; section III. N. Transfer and Release of Detainees.
- PBNDS 2011: Medical Care; section II. Expected Outcomes.
- PBNDS 2011: Medical Care; section V.J. Medical and Mental Health Screenings of New Arrivals.
- PBNDS 2011: Medical Care; section V. U. Delivery of Medication.

3. Documentation

- On February 2, 2020, an ECJ medical staff member (credential unknown) ordered twice daily blood sugar checks for three days. The medical staff member did not document an assessment of Mr. PEREZ.
- On February 11, 2020, a WCC RN did not document an assessment of Mr. PEREZ upon his return from the hospital.
- Between February 11 and 26, 2020, WCC housed Mr. PEREZ in the MHU. WCC nursing staff did not consistently document a nursing assessment.
- Between February 11 and 26, 2020, WCC nursing staff did not consistently assess and monitor Mr. PEREZ according to his MHU orders.

~~For Official Use Only. This document contains pre-decisional and/or deliberative process information exempt from mandatory disclosure under the Freedom of Information Act, 5 U.S.C. 552(b) (5). Do not release without prior approval of U.S. Immigration and Customs Enforcement, ICE Health Service Corps.~~

Mortality Review –

Sanchez PEREZ, Luis (aka HERNANDEZ-Cabrera, Mauricio), (b)(6),(b)(7)(C)

Page 8 of 9

- On February 12, 2020, a WCC APP ordered BP checks every eight hours; however, the WCC nursing staff completed the checks twice daily.
- On February 12, 2020, a WCC APP ordered strict intake and output measurements for Mr. PEREZ; however, WCC nursing staff only completed this order on February 17, 2020, and did not document the results.
- On February 13, 16, 20, 21, 22 and 23, 2020, WCC nursing staff did not document reassessment of Mr. PEREZ's BP and associated symptoms after administering clonidine.
- On February 24, 2020, an APP added daily weight checks to Mr. PEREZ's MHU monitoring orders. The WCC nursing staff did not consistently document completion of the new order.

Applicable standards of care for this finding:

- PBNDS 2011: Medical Care; section II. Expected Outcomes.
- PBNDS 2011: Medical Care; section V. BB. Medical Records.
- Louisiana Nurse Practice Act; Chapter 39. Legal Standards of Nursing Practice, 3901, 3903, 3905, and 3909. September 2020.

4. Communication

- On February 11, 2020, Mr. PEREZ returned to WCC with significant medical diagnosis; however, the WCC medical department did not notify ICE of his medical condition until February 21, 2020.
- On February 25, 2020, the prior WCC health services administrator documented in Mr. PEREZ's medical record she informed ICE of the APP and MD's agreement to clear Mr. PEREZ for travel to Guatemala. During the remote site visit, the APP stated he cleared Mr. PEREZ for transfer to a higher-level of care facility, not deportation to Guatemala.
- On February 26, 2020, CCC did not receive prior notice of Mr. PEREZ's transfer or the acuity of his condition.

Applicable standards of care for this finding:

- PBNDS 2011: Medical Care; section V.N. Medical/Psychiatric Alerts and Holds.
- PBNDS 2011: Medical Care; section V.X. Notification of Detainees with Serious Illness and Other Specified Conditions.
- PBNDS 2011: Medical Care; section V.Z. Continuity of Care.
- PBNDS 2011: Medical Care; section V. BB. Medical Records.

Mortality Review –

Sanchez PEREZ, Luis (aka HERNANDEZ-Cabrera, Mauricio), (b)(6),(b)(7)(C)

Page 9 of 9

5. Training

- On February 7, 2020, a WCC RN completed Mr. PEREZ's health assessment without the appropriate training.

Applicable standards of care for this finding:

- PBNDS 2011: Medical Care; section V. M. Comprehensive Health Assessment.

Recommendations

- Forward these findings to the IHSC Deputy Assistant Director of Health Care Compliance (DAD-HCC).
- The IHSC DAD of HCC will share these findings through appropriate communication channels to ICE, ECJ, and WCC's administrator and health authority for review and any action(s) deemed appropriate.
- The respective IHSC HCC unit and ICE will ensure the corrective action plan, if applicable, is implemented and sustained.

End of report.